

# Skin Foreign Body - Female (Adult)

Algorithm 1266 | Adult (18 years and older) | Gender at birth: Female

## Definition

- Qatar-localized UAT-only pathway for a suspected splinter or embedded skin object in an adult or child; it screens for bleeding, neurovascular injury, infection, high-risk sites and safeguarding concerns and is not approved for production.
- Adult patient.
- Female.

## Initial assessment

1. [LOCATION] Where is the object?
2. [OPEN\_TEXT] What is the object, and how deep is it?
3. [YES\_NO] Has removal already been attempted?
4. [OPEN\_TEXT] What is the exact age; is the object near an eye, face, neck, chest, abdomen, genital area, joint, tendon, blood vessel or nerve; and are there diabetes, poor circulation, immune suppression, pregnancy, tetanus concerns, possible deliberate injury or an inconsistent history?
5. [YES\_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?

## Triage and redirect questions

### **1. Redirect → Puncture Wound**

Is this primarily a narrow puncture wound with no retained object?

A narrow penetrating wound without a retained object belongs in the Puncture Wound pathway.

### **1. Disposition level 100**

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are GOVERNANCE\_REQUIRED.

### **2. Disposition level 78**

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway is GOVERNANCE\_REQUIRED.

### **3. Disposition level 100**

Is the object large or deep, is bleeding uncontrolled, or is there loss of feeling or trouble moving near it?

NHS.UK cuts and grazes guidance lists an embedded object with uncontrolled bleeding or nerve involvement as an emergency-assessment criterion.

### **3. Disposition level 70**

Is the object made of glass or metal and deeply embedded, or is the skin around it hot, red, swollen, or leaking pus after a removal attempt?

Deeply embedded glass/metal or signs of infection after removal warrant professional removal and review rather than repeated home attempts.

### **3. Disposition level 70**

After emergency features are excluded, is any skin foreign body still present or suspected, including a small or shallow splinter?

This UAT pathway requires an in-person assessment before removal advice because depth, material, retained fragments, tetanus status and pediatric or safeguarding risk cannot be reliably excluded remotely.

## Advice

### **Immediate child safeguarding danger**

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are GOVERNANCE\_REQUIRED. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

### **Urgent private child safeguarding assessment**

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar

child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact reporting, consent, documentation, and information-sharing rules are GOVERNANCE\_REQUIRED.

- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

### ***Emergency embedded object precautions***

- Call Qatar 999 for uncontrolled bleeding, collapse or severe neurovascular symptoms and do not drive. Do not remove the object. Apply pressure around, not on, an embedded object while awaiting help. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: bleeding will not stop; numbness, pallor or weakness spreads.

### ***Urgent foreign body review***

- Arrange same-day in-person review through the Qatar pathway approved for this UAT environment. Do not dig, squeeze or repeatedly attempt removal. A clinician must assess retained fragments, wound care and tetanus needs.
- Call Back If: increasing redness or pain; fever or reduced movement develops.

### ***In-person review for a small skin foreign body***

- Keep the area clean, avoid squeezing or probing it, and arrange same-day in-person assessment through the locally approved Qatar UAT pathway. Do not give medication advice until age, weight, pregnancy, allergy, kidney/liver disease and interactions are checked.
- Call Back If: cannot remove it; bleeding, numbness or signs of infection develop.

## **First aid**

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are GOVERNANCE\_REQUIRED. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Qatar 999 for uncontrolled bleeding, collapse or severe neurovascular symptoms and do not drive. Do not remove the object. Apply pressure around, not on, an embedded object while awaiting help. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

## **References**

- NHS.UK, "Cuts and grazes", <https://www.nhs.uk/conditions/cuts-and-grazes/> (page last reviewed 02 April 2026) - embedded object criteria; standard first-aid splinter-removal technique (not a direct NHS.UK quote)

## **Provenance**

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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